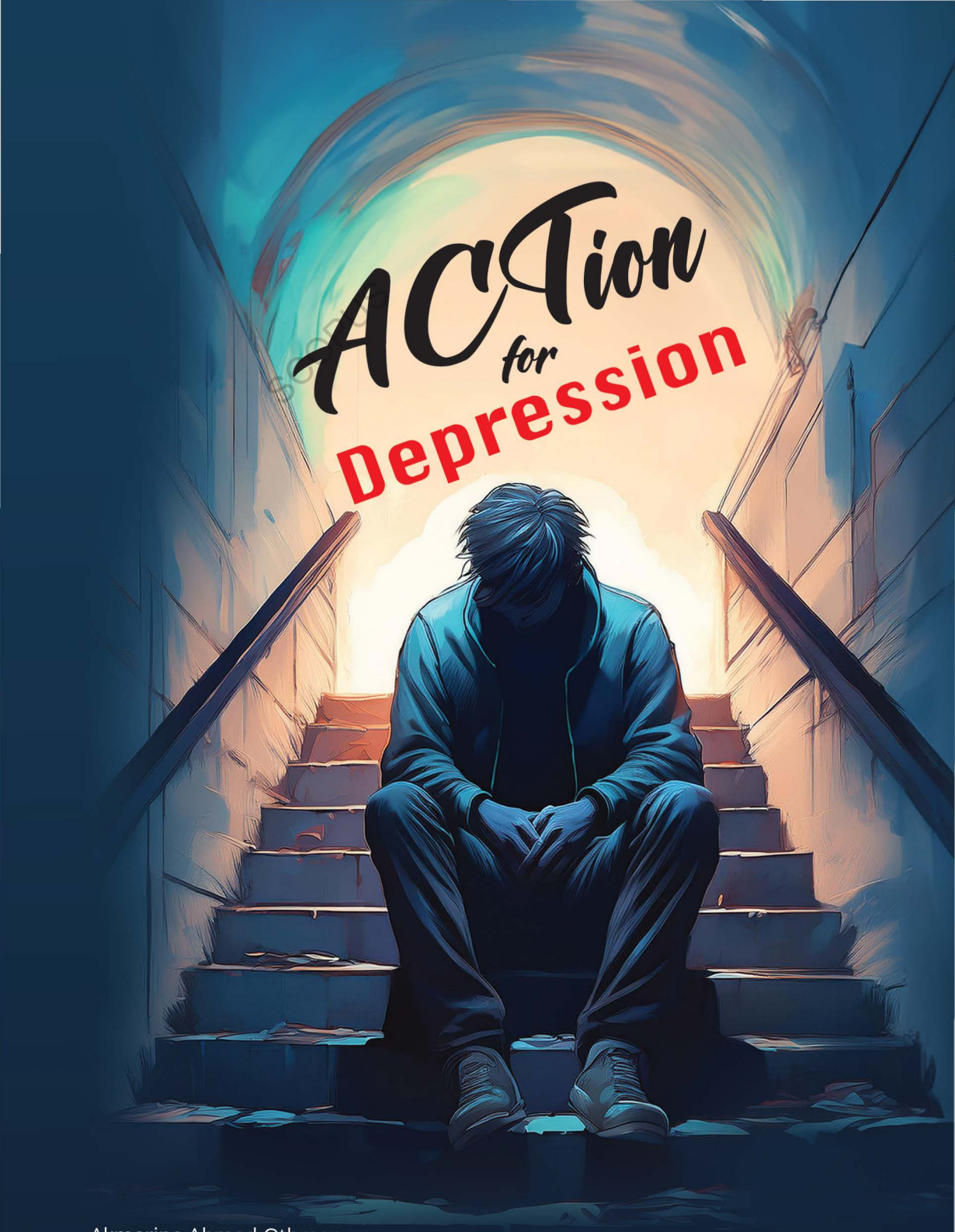
ACTion for Depression



Akmarina Ahmad Othman
Wan Marzuki Wan Jaafar
Zaida Nor Zainudin
Yusni Mohamad Yusop

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ACTion for Depression

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Akmarina Ahmad Othman
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Contents

Preface vii
Acknowledgement ix
Abbreviations xi

Chapter 1 Introduction on ACTion for Depression 1
Chapter 2 History of ACT and Depression 4
Chapter 3 Going Down the Lane 11
Chapter 4 ACTion for Depression 20
Chapter 5 ACTion Overload and other Pitfalls 40
Chapter 6 Brief ACTion Planning 43
Chapter 7 Wrap-up 49

References 51
Biodata of Authors 61
Index 63

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Preface

This book discusses how Acceptance and Commitment Therapy (ACT) can be used to overcome depression. Throughout the book, the term ACT will be used to refer to Acceptance and Commitment Therapy.

This book is designed for a broad audience, ranging from counsellor and mental health professional who have never been exposed to ACT before to those who are experienced. Given that the primary audience is counsellor, the term "client" is used throughout instead of "patient," as it is more commonly used in counselling settings. The book is suitable for individuals who wish to improve their skills in assisting clients to build psychological flexibility by employing a variety of techniques used to achieve desired outcome process in ACT. ACT is not only addresses immediate symptoms of depression but also promotes long-term psychological flexibility and resilience.

The book describes the theories related to ACT and Depression, the process and techniques used in formulating client's case, as well as guiding counsellors in planning the treatment as well as structuring the sessions.

The book is hoped to serve as a valuable resources and references for both professionals and individuals. Our goal is for readers to gain a deeper understanding of how ACT can be effectively used to manage and overcome depression.

Akmarina Ahmad Othman
Wan Marzuki Wan Jaafar
Zaida Nor Zainudin
Yusni Mohamad Yusop
Serdang, 2024

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This book would not have been possible without the support, encouragement, and guidance of many individuals.

First and foremost, we want to express our deepest gratitude to all respondents who took part in Acceptance and Commitment Therapy. Your support and contribution along the journey are highly appreciated.

A heartfelt thank you to my husband, my children, my mother, my family, and my family in-law whose constant encouragement and understanding provided me with the strength to keep going. Your love and support have meant the world to me.

We would also like to acknowledge Penerbit Universiti Putra Malaysia for their meticulous attention to detail and for helping to bring this manuscript to life. Your feedback and suggestions greatly improved the quality of this work.

To our colleagues and friends at Universiti Putra Malaysia, thank you for your constructive criticism, stimulating discussions, and for always being there to share in both the struggles and successes.

Finally, we want to thank my readers. Your interest and enthusiasm inspire us to keep writing, and we hope this book resonates with you in a meaningful way.

Thank you all.

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List of Abbreviations

ACT	Acceptance and Commitment Therapy
CH	Creative Hopelessness
MDD	Major Depressive Disorder
RFT	Relational Frame Theory
SAC	Self-as-context
UPM	Universiti Putra Malaysia

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1

Introduction on ACTion for Depression

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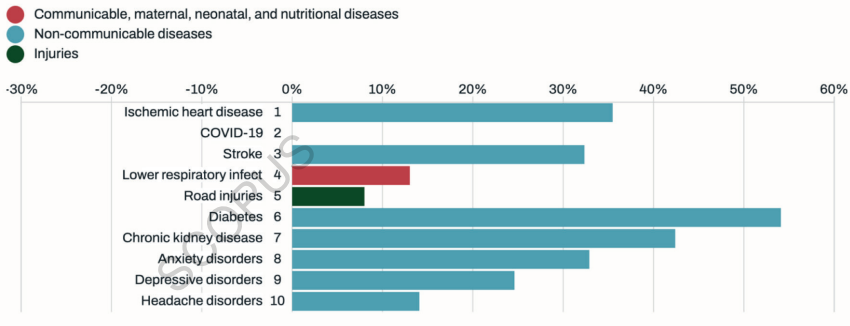
DEPRESSION

Depression is a mental disorder that has impacted over 280 million people worldwide (World Health Organization, 2021). The number of adults with mental health issues in Malaysia has increased from 10.7% in 1996 to 29.2% in 2015 (Institute for Public Health, 2015). Reportedly, females, younger adults, other Bumiputras, and adults from low-income families are highly susceptible to mental health problems. The rising number of depressed young adults has garnered much scholarly interest as this issue could induce poor functioning at work and home (World Health Organization, 2021), chronic health ailments, and low productivity and living standards (Gibb et al., 2010; Gili et al., 2018). People with major depressive disorder (MDD) may experience internal suffering that subsequently induces poor work and home functioning (World Health Organization, 2021). In Malaysia, MDD is listed as one of the top 10 illnesses that increases Disability-adjusted Life Years (DALYs) in 2021 (Institute for Health Metrics and Evaluation, 2021) (Figure 1.1). This rising trend necessitates prevention, early screening, and awareness of depression for timely and appropriate treatments.



Source: Institute for Health Metrics and Evaluation. (2021). *Malaysia profile*. IHME, University of Washington. http://www.healthdata.org/Malaysia

Figure 1.1 Top 10 Causes of Death and Disability (DALYs) in 2021 and Percent Change 2011–2021, All Ages Combined.

Psychological problems are commonly higher among adolescents from 16 to 19 years old (34.7%), followed by those aged 20 to 24 (32.1%), and 25 to 29 (30.5%) (Institute for Public Health, 2015). Young adults aged 18 to 29 is the most volatile life stage compared to other phases following fluctuations in relationships, educations, family, and employment which leads to instability (Arnett, 2000). In agreement with this, Gustavson et al. (2018) states major depression and anxiety disorders are highly prevalent among young adults from 18 to 29 years old. This prevalence subsides in their 30s and 40s. Furthermore, an individual's risk of developing another mental condition a decade later is increased if they were already diagnosed in their 20s. Such occurrences could impact young adults' capacity to pursue education, establish a family, and become employed.

Reportedly, depression, anxiety, and stress are the most common mental health issues among university students (Merikangas et al., 2009; Mustaffa et al., 2014; Nigatu et al., 2016). Cuijpers et al. (2016) concurred with this report as university and college students are highly vulnerable to develop mental health problems. Such students, who fall in the age range when common mental health problems are at a developmental peak, frequently encounter high stress levels. Timely and appropriate interventions prove necessary given the high susceptibility of young adults to develop mental health problems. Depression and anxiety in Malaysia have increased from 2013 to 2019. There is a high prevalence of undergraduate studies who

experienced moderate to severe depression (21%), anxiety (50%), and stress (12%) (Amir Hamzah et al., 2019, Balami et al., 2014; Mohammed et al., 2016).

Psychological intervention efficacy on Malaysian young adults' depression remains scarce. Following past works, there is lack of intervention focusing on young adults with depression, specifically in Malaysia. It is deemed vital to investigate this group as features entailing instability and self-focus render them susceptible to depression and anxiety (Arnett et al., 2014; Kulig & Persky, 2017; Pettit et al., 2011). Moreover, their high identity exploration and negativity levels also render them vulnerable to psychological stress (Baggio et al., 2017).

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ACCEPTANCE AND COMMITMENT THERAPY FOR DEPRESSION

Acceptance and Commitment Therapy (ACT) is a psychotherapy intervention following the Relational Frame Theory (RFT), is commonly referred to as 'third wave' of behavioural therapies (Hayes et al., 2001). Rather than emphasising symptom reduction (e.g., anxiety and negative thoughts), ACT encourages participants to perform value-based actions regardless of whether they are experiencing symptoms or otherwise (Harris, 2019). Notably, ACT works with depressed individuals to encourage their psychological flexibility rather than eliminating depression (Hayes et al., 2004). Self-as context, defusion, acceptance, contact with the present moment, values, and committed action are the six core therapeutic processes or known as hexaflex that contribute to psychological flexibility: the ability to "be present, open up, and do what matters." The ACT successfully demonstrated its effectiveness in treating undiagnosed or sub-clinical depression (Heydari et al., 2018; Ito & Muto, 2020). Therefore, this book is going to focus on the use of ACT in treating depression among young adults. Even so, this book can be used as a reference for other age group as well.

2

History of ACT and Depression

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BECK'S COGNITIVE THEORY OF DEPRESSION

Aaron T. Beck's establishment on depression has an intriguing background. He began investigating all his depressed patients' dreams and stated that the dreams would manifest greater themes of hostility than the non-depressed patients. Surprisingly, more themes of deprivation, loss, and defectiveness were identified compared to hostility. Furthermore, when the patients were awake, their thoughts resembled these themes. This rises the psychoanalytic idea where depressed people subject themselves to suffering might be inaccurate (Beck, 1967).

People with depression think differently compared to their undiagnosed counterparts. For example, two different people (one who is depressed and another who is not) facing the same situation would generate distinctive thoughts, behaviours, and emotions. Depressed people's dysfunctional beliefs coupled with stressful life events would produce negative thoughts, which is the root cause of their depression symptoms (Beck et al., 1979).

Beck assumed three specific concepts to describe the psychological elements (cognitive triad, schemas, and cognitive distortions) underlying depression. Cognitive triad consists of three forms of negative thinking or known as negative automatic thoughts that are typical in depressed individuals: a negative view of the self, a negative view of the world, and a negative view of the future. Such people tend to demonstrate this cognitive triad when automatic thoughts render themselves inadequate, defective, and deprived (Haaga et al., 1991). Individuals with depression also perceive the world to be against them, misinterpret their life obstacles

or experiences, and anticipate the current challenges to persist indefinitely and cause failure.

The schema concept denotes that similar situations might be conceptualised by different persons in different ways. Nevertheless, people tend to have consistent responses to similar types of events. This consistency develops a relatively stable cognitive pattern, which underpins the regularity of interpretations in a particular set of situations. A schema is the basis for moulding and designing these stable cognitive patterns, would be activated in a specific situation to determine individual responses. In a depressed person, his conceptualisations of specific situations are distorted. Over times or in more severe cases of depression, their cognitive organisation may become independent of external stimulation and render the person unresponsive to the changes surrounding his environment.

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The third component, cognitive distortions or faulty information processing, denotes the systematic errors in depressed people's thinking that maintain their beliefs despite contradictory evidence (Beck et al., 1979). Beck and colleagues (1979) denoted such cognitive distortions as arbitrary inference, selective abstraction, overgeneralisation, magnification and minimisation, personalisation, and dichotomous thinking. The aforementioned distortions are systematic errors in reasoning that induce faulty assumptions and misconceptions.

RELATIONAL FRAME THEORY UNDERLYING ACT

ACT interventions rely on Relational Frame Theory (RFT) principles to influence the contextual and functional features of language and cognition. These principles are directly applicable to the development of human pain, psychopathology, and clinical interventions. It is practically ineffective to isolate human language and cognition functions from the context in which the human organism operates. The RFT characterises language and cognition as a form of relational behaviour, which allows the human organism to link events bidirectionally and integrate them. Meanwhile, direct experience is only a one-way phenomenon. For example, an individual who sees a chair would think of the word 'chair' and subsequently picture an actual chair (Fletcher & Hayes, 2005).

From the RFT perspective, the failure to separate the continuous thought process from the product of thinking (thoughts) has caused the

derived relations to control other sources of behavioural regulations, which leads to psychopathology. This occurrence is known as cognitive fusion, where one's cognition supersedes physical or psychological behaviour in a self-defeating or disruptive manner (Harris, 2019). Cognitive fusion causes three implications that are interrelated between RFT and ACT. The first side effect occurs when internal events are associated with temporal and evaluative connections and people learn to anticipate, fear, and strive to regulate and avoid personal thoughts, feelings, and bodily sensations even when this process is harmful. This situation is known as 'experiential avoidance' (Hayes et al., 1996), which is the perpetual struggle to avoid undesirable thoughts and emotions (Harris, 2019). Second, people become deeply immersed in their own self-descriptions and strive to maintain and justify them. The third effect implies the dissolution of the present moment into a mixture of human thoughts, reasoning, explanation, and justifications for behaviour or 'psychological rigidity': the inability to maintain or change behaviour in alignment with chosen values.

The six core pathological processes of psychological rigidity containing fusion, experiential avoidance, inflexible attention, remoteness from values, unworkable action, and fusion with self-concept further explain this situation (Harris, 2019). These processes are actually the inversed form of the six core therapeutic processes in ACT: defusion, acceptance, present moment, values, committed action, and self-as-context. The six core ACT processes are not separate, but rather resemble the six facets of a diamond. Meanwhile, psychological flexibility, which is the ability to "be present, open up, and do what matters" (Figure 2.2) (Harris, 2019) represents the diamond itself (Figure 2.1).

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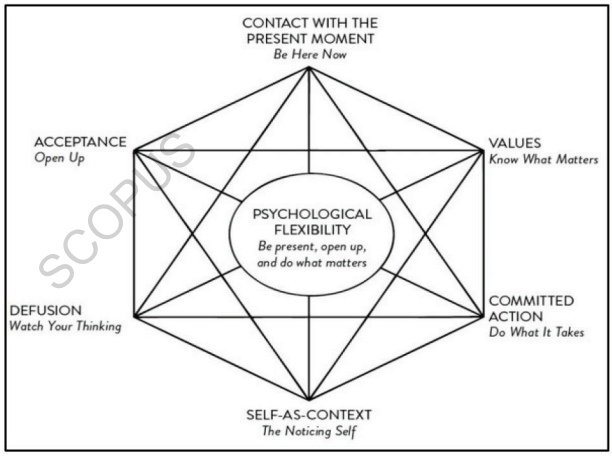

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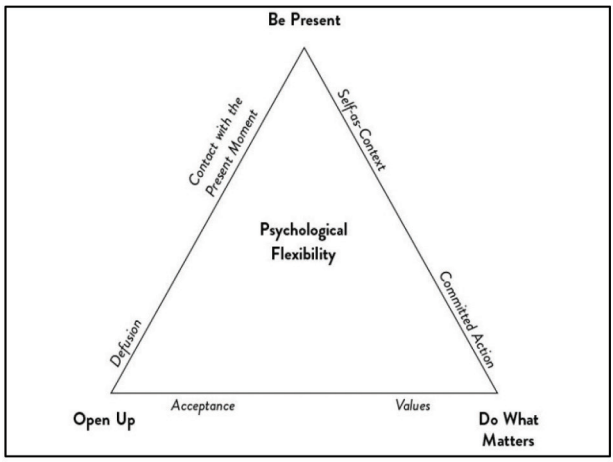
Source: Harris, R. (2019). ACT made simple: An easy-to-read primer on acceptance and commitment therapy. New Harbinger Publications.

Figure 2.1 The ACT Six Core Processes



Source: Harris, R. (2019). *ACT made simple: An easy-to-read primer on acceptance and commitment therapy*. New Harbinger Publications.

Figure 2.2 The ACT Triflex

Contacting the present moment is defined as adjusting flexibly to what we are experiencing at the moment by adequately narrowing, widening, moving, or retaining the concentration. *Self-as-context* denotes "the part of you that notices", which implies part of us that aware of whatever we think, feel, or do at any moment. Both these core processes constitute the 'be present' functional unit of ACT triflex (see Figure 2.2). The next core process is *defusion*, which is learning to step back and detach from personal thoughts, images, and memories. Next is *acceptance*, which entails acknowledging and allowing space for unwanted personal experiences, such as thoughts, feelings, memories, desires, and sensations. Defusion and acceptance combined together to become 'open up' part of the ACT triflex. The third and last part of ACT triflex implies 'doing what matters' based on *values* and *committed action*. Values, which denote desired physical or psychological qualities, describe how one intends to act throughout his life. Committed action involves the performance of physical or psychological actions that are guided by personal values, establishing goals, action-planning, solving problem, developing skills, behavioural activation, and exposure. Table 2.1 shows the summary of the core therapeutic processes and their definition. Meanwhile Table 2.2 describes on the summary of ACT Triflex**.**

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Table 2.1 Core therapeutic processes and their definition

Core Therapeutic Processes	Definition
Contacting the present moment	Mindfully attending to our current experience with flexibility: narrowing, widening, moving, or maintaining concentration, depending on what is most beneficial.
Self-as-context	It is "the part of you that notices" or "noticing self". It is that part of us that is mindful of whatever we think, feel, or do at any moment.
Defusion	Learning to "step back" and create some distance from our thoughts, images, and memories or disengage from them.

Table 2.1 *cont'd*

Core Therapeutic Processes	Definition
Acceptance	Opening up and making room for personal experiences that are unwanted: thoughts, feelings, memories, desires, images, and sensations. We open up and make space for them instead of fighting, resisting, or running from them.
Values	Wanted qualities of physical or psychological action. On an ongoing basis, they describe how we want to act.
Committed action	Taking purposeful action based on our values, which involves both physical actions (what we do with our bodies) and psychological actions (what we do internally). This includes setting goals, action planning, solving problems, developing skills, and engaging in behavioral activation.

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Table 2.2 The ACT Triflex

Functional units	Core processes	Definition
Be present	Contacting the present moment Self as context	Paying attention to your here-and-now experience flexibly and engaging it.
Open up	Defusion Acceptance	Distancing ourselves from thoughts and emotions, recognizing them for what they truly are, allowing space for them, and letting them come and go naturally.
Do what matters	Values Committed Action	Setting up and maintenance of life- enhancing action.

In simple words, ACT is about "mindful" action: action that we take intentionally, open to our experience, with complete consciousness, and participating in everything we do. The aim of ACT is to identify our values and use this understanding to guide, motivate, and empower us to engage in activities that improve and enrich our lives. Additionally, it

teaches psychological skills (such as 'mindfulness') that help us manage challenging thoughts and emotions, fully participate in our activities, and savour the fulfilling aspects of life.

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Going Down the Lane

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YOUNG ADULTS AND DEPRESSION

Young adults are defined as those between 18 to 29 years old and they are in the age range of exploration involving various life aspects, such as love, relationship, and work (Arnett, 2014). They are also described as instable, which characterises by a highly volatile period due to frequent shifts in love, relationships, and work compared to other life stages (Arnett, 2000). Young adults would independently (voluntarily) choose their life paths and options. Regardless, such decision-making and life-altering options could be involuntary in certain cases, such as failed examinations and relationships and work termination. Such involuntary life issues could induce instability and pervasive emotions, such as depression and anxiety (Arnett et al., 2014). Instability in young adults is also associated with moving across places: a common occurrence among this population as most of them are transitioning from secondary to tertiary education and work settings (Kulig & Persky, 2017). As moving to a new location could prove stressful to young adults (Kulig & Persky, 2017; Zarate, 2010), they require optimal social support to accommodate their new surroundings. Arnett and Schwab (2012) works on highlighting that approximately 51% of young adults heavily rely on friends and family for social support.

Young adults are the most self-focused individuals with minimal obligations and responsibilities to others (Arnett, 2000). They would still encounter parental authority, albeit not as high as when they were under 18 years old. They may also not be in a long-term relationship. Self-focus induces low social support and depression (Arnett et al., 2014). Likewise,

Pettit et al.'s (2011) found relations between depression symptoms and social support in young adulthood. An inversed version of this relationship was also identified, with depression symptoms being highest in the individuals' early 20s and decreasing with age. Meanwhile, social support was at its lowest in their early 20s and increased with age. Hence, self-focus that causes low social support may increase depression symptoms among young adults.

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Moreover, young adults also view themselves as neither adolescents nor adults. In other words, the individuals are transitioning to adulthood (Nelson & Luster, 2015). Three criteria are considered to regard an individual as an adult: accepting self-responsibility, making independent decisions, and gaining financial independence (Arnett, 2014). As these benchmarks occur gradually, Arnett et al. (2014) denoted that feeling in-between might induce depression and anxiety among young adults, specifically those who feel obligated to behave like adults at their current age. Reportedly, most young adults feel anxious while almost one-third experience depression (Arnett & Schwab, 2012).

Ahmad Othman et al. (2024) shows that ACT is more effective than psychoeducation to reduce depression symptoms among young adults and this is being supported by Dereix-Calonge et al. (2019), Grégoire et al. (2018), Ito and Muto (2020), Ruiz et al. (2020), and Zemestani and Mozaffari (2020). Although it is being demonstrated that depression level is reduced in psychoeducation, the decrease is insignificant. Furthermore, symptoms in the psychoeducation worsened during the three-month follow-up compared to ACT, where the depressive symptoms reduced more.

YOUNG ADULTS, DEPRESSION, AND MALAYSIA

Shamsuddin et al.'s (2013) reported that there is a high prevalence of depression, anxiety, and stress symptoms of moderate severity and above, which were 37.2%, 63.0% and 23.7%, respectively. Depression, anxiety, and stress proved significantly higher among students from 20 to 24 years old compared to their younger and rural-area counterparts. Perceivably, older students might be concerned about their future in terms of employment, relationships, and fear of failure (Shamsuddin et al., 2013). Likewise, Latiff et al.'s (2014) work involving undergraduate students between 19

and 25 years old revealed a high prevalence of depression, anxiety, and stress symptoms at 27.3%, 60.0%, and 22.6%, respectively. The associated factors of depression encompassed the male gender, Malay ethnicity, body image concerns, and religious personalities.

Depression, anxiety, and stress among university students have steadily increased over time. Mohammed et al.'s (2016) revealed the prevalence of depression, anxiety, and stress at 32.1%, 64.6%, and 29.2%, respectively, and their associated factors among first-year undergraduates in Malaysia. Additionally, the students' common coping mechanisms include religious coping, positive reframing, and planning. Some students also used avoidant coping strategies, such as self-distraction, venting, and self-blame. Despite the utilisation of active rather than avoidant coping strategies, the small number of students using the latter could not be disregarded following the risk factors for adverse responses to depression and anxiety (Mohammed et al., 2016). The factors associated with depression entailed gender, age, study major, parents' education and occupation, whereas the risk factors associated with anxiety were study major and parents' education. Amir Hamzah et al.'s (2019) finding on the prevalence of depression, anxiety and stress among first-year undergraduate students ranging from 18 to 25 years old highlighted the high prevalence of depression, anxiety and stress at 21%, 50%, and 12%, respectively. Justifiably, students' relocation from home to university, adjustment to unfamiliar surroundings, and interaction with new people could be emotionally taxing and stressful.

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ACTion ON DEPRESSION AMONG YOUNG ADULTS

ACT has garnered much popularity in treating depression and helping clients accept painful internal experiences (thoughts, emotions, and sensations) to engage in value-based behavioural changes. Meanwhile, other therapy such as CBT employs cognitive disputation and other forms of reappraisal to fix biases in information processing with the aim of reducing symptom severity (Beck et al., 1979).

University students are among the largest population in young adults. Chinese international students at public universities in the USA demonstrated that a brief two-session ACT substantially minimised depression, anxiety, and stress levels (Xu et al., 2020). Danitz et al. (2016) similarly performed an acceptance-based behavioural therapy (ABBT)

programme on undergraduate students and the participants significantly recovered from depression over the semester. In addition to that, another four weekly ACT interventions among students demonstrated an increased in well-being and school engagement and reduced depression, anxiety, and stress symptoms (Grégoire et al., 2018). In Japan, ACT was shown to be effective in reducing depressive symptoms among undergraduate university students with symptoms of irritable bowel syndrome (Ito & Muto, 2020). The ACT successfully demonstrated its effectiveness in treating depression symptoms, whether in clinically diagnosed major depressive disorder (Davoudi et al., 2017; Far et al., 2017; Zemestani & Mozaffari, 2020), anxiety disorder (Ruiz et al., 2020), or obsessive-compulsive disorder (Twohig et al., 2018; Vakili et al., 2014).

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ACTion ON QUALITY OF LIFE AMONG YOUNG ADULTS

ACT strives to optimise functioning in multiple valued domains rather than symptom reduction. Thus, ACT may be able to offer a wider approach to behaviour change and high quality of life (Hayes et al., 1999). Similarly, Dalrymple et al. (2014) disclosed that depression symptoms improved along with the improvement of quality of life in ACT. Unlike other treatments (e.g., Cognitive behavior therapy, CBT) that typically emphasise symptom reduction, ACT still denotes a significant improvement in this component following the transition to value-congruent behaviours. Dalrymple et al. (2014) implied that behavioural changes and increased emotional awareness led to perceived improvement.

Ducasse et al. (2018) demonstrated a significant improvement in quality of life after ACT intervention. The participants were taught on mindfulness and in result they were more optimistic about and content with life (Brown & Ryan, 2003). Dalrymple et al. (2014) reflected enhanced quality of life following a 16-week session, whereas Ducasse et al. (2018) revealed the same finding in a 7-week session. Dindo et al. (2020) reported an optimal quality of life after a one-day ACT. This shows that ACT is effective in improving the quality of life.

The measurement of quality of life is important in dealing with ACT following its emphasis on improving functioning and quality of life rather than merely perceiving symptom reduction (Hayes et al., 1999). In Zimmerman et al. (2006), depressed patients rated aspects other than

symptom reduction as crucial in assessing the extent to which they had achieved remission (a restoration to original functioning in terms of physical and mental health). Similarly, Saarni et al. (2007) conceded that psychological therapies and other treatment for mental illness should not just rely on symptoms as targets and results, but also the larger concept of quality of life.

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ACT VS. CBT

Traditional CBT is not a new therapy among mental health professionals. This therapy has been successfully applied over many decades. Meanwhile, ACT is a more recent therapy that has gained much popularity. Comparison between CBT and ACT among young adults ranging from 18 to 29 years old remain underexplored as most emphasised adults aged 18 years old and above (A-Tjak et al., 2021; Kocovski et al., 2013; Pleger et al., 2018; Samaan et al., 2021).

Forman et al. (2007), who compared ACT and CBT, revealed that the participants in both conditions substantially reduced depression, anxiety, and quality of life. Although both interventions proved equally effective in reducing depression and anxiety symptoms and improving quality of life, the changes in the investigated mediators were highly associated with improved treatment in both ACT and CBT. Changes in 'noticing' and 'describing' one's experiences mediated the CBT group effects, while 'experiential avoidance', 'behaving with awareness', and 'acceptance' are the mediators for the ACT group. A one and a half years follow-up was done and shows more CBT patients depicted higher treatment maintenance for depression and quality of life during follow-up, but not for anxiety. Several reasons underpinned these result, For example, CBT is a more simplified intervention that renders it convenient for patients to use it and apply the learnt skills post-treatment. Furthermore, most of the participants were high-functioning with low levels of symptoms. From some expert views, acceptance-based psychotherapies proved more suitable for treatment-resistant patients with severe symptoms (Kenny & Williams, 2007).

Niles et al. (2014) demonstrated that a higher reduction in negative cognitions at the beginning of the treatment predicted a better reduction in social anxiety symptoms and depression for both CBT and ACT post-treatment. Regardless, this did not apply to experiential avoidance with its

rate of change at the start of the treatment only specific to ACT rather than CBT. In other words, a faster decline in negative cognitions at the start of therapy predicted overall improvement, whereas a faster drop in experiential avoidance only predicted ACT (not CBT) improvement. Another comparison between CBT and ACT was done in female undergraduate students diagnosed with MDD by Tamannaeifar et al. (2014). Both the ACT and CBT participants demonstrated a significant reduction in depression symptoms following 12 therapy sessions. Although this shows the same effectiveness between ACT and CBT, specific limitations should be highlighted. Particularly, only female students and a small number of participants were used in the comparison.

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Talking about adults in general, ACT was compared against CBT on pain interference, depression, and pain-related anxiety in adults with chronic pain conditions (Wetherell et al., 2011) and both were effective in reducing all the three symptoms. Similar to Wetherell et al. (2011), Fischer et al. (2021) examined depression, anxiety, and quality of life in patients with chronic pain using CBT and ACT, and similar finding was reported where CBT and ACT proved to be effective in lowering depression and anxiety levels and improving quality of life. Burian et al.'s (2021) investigation on ACT and CBT involving depression and anxiety symptoms among patients with psychiatric diagnoses (depression and somatoform and anxiety disorders) showed similar effectiveness in daily functioning, quality of life, and symptom reduction in depression and anxiety. It shows that patients with mental and physical health issues in routine care hospital settings may benefit from ACT-based group treatment apart from CBT.

ACT and CBT were also being compared in dementia family caregivers with depression symptoms (Losada et al., 2015) and both therapies showed a significant reduction in depression symptoms following eight sessions. Regardless, the symptoms were only maintained in CBT during a six-month follow-up. This result similar with past study that used young adults and revealed higher maintenance during follow-up for depression and quality of life in CBT compared to ACT (Forman et al., 2012). One possible reason underpinning these is participants' perceptions of the difficulty in practising and applying the learnt skills. Although no significant differences were highlighted for the homework quality, ACT participants reported homework assignments to be more challenging

than those in CBT (Losada et al., 2015). Following Forman et al. (2012), CBT is a relatively simplified and intuitive intervention that complements folk psychology, thus rendering it easier for patients to implement and generalise the acquired skills in daily life. Contrarily, ACT necessitates individuals to grasp abstract concepts (e.g., cognitive defusion and experiential avoidance) and involves techniques and exercises that require a high degree of metacognitive abilities. Resultantly, ACT may demand more time and effort from patients and high therapist support to understand its rationale, learn the techniques, and apply them in the long run. Romero-Moreno et al. (2021) utilised a similar group of participants to investigate the influence of baseline depression and anxiety comorbid symptoms on treatment responsiveness among dementia caregivers. Caregivers with baseline clinical depression and anxiety comorbid symptoms were less effective with CBT and ACT therapies compared to those without baseline comorbidity. Thus, baseline depression and anxiety comorbidity may hamper the treatment response.

Pleger et al. (2018) investigated inpatients with mood disorders as the primary diagnosis and both CBT and ACT depicted a similar and significant reduction in depressive symptoms. This finding is supported by Samaan et al. (2021) who concluded that CBT and ACT are equally effective in treating individuals with depression in a routine clinical setting and stabilising the symptoms over a six-month follow-up. Samaan et al. (2021) also suggested that patients with severe symptoms may struggle to engage in ACT. Nevertheless, Kenny and Williams (2007) opposed this statement as acceptance-based psychotherapies were deemed more suitable for treatment-resistant individuals with severe symptoms.

Besides that, both interventions significantly reduced depressive symptoms and improved quality of life over six months among participants with MDD (A-Tjak et al., 2018). Although different treatments may generate similar results, there is still debates on the necessity of new treatments for depression. Possibly, some patient characteristics, such as patient's preferences increase the likelihood of successful treatment with over another (Kwan et al., 2010). The presence of multiple treatment alternatives may benefit both patients and therapists. A-Tjak et al. (2018) also suggested ACT application to prevent the initial onset or recurrence of depression.

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Ahmad Othman et al. (2024) shows no differences between ACT and CBT in depression level, which means both are effective in reducing depression and this reduction was maintained at three-month follow-up. Figure 3.1 depicts the mean of depression before and after therapy, as well as at follow-up for ACT, CBT, and psychoeducation group in the study by Ahmad Othman et al. (2024). This is similar with Avdagic et al. (2014), A-Tjak et al. (2021), A-Tjak et al. (2018), and Samaan et al. (2021). However, despite being non-significant, ACT reflected a higher reduction in depression compared to CBT at after the therapy and during follow-up. Likewise, Talaeizadeh (2020) conceded that ACT depicted a more significant reduction in depression compared to CBT. One of the reasons underlying high ACT influence on depression is the application of techniques to increase psychological flexibility: mindfulness exercise, acceptance, and defusion skills. These techniques and the identification of personal values may reduce depression symptoms (Talaeizadeh, 2020). Values are one of the ACT components that may be useful for young adults in their crucial transitional period where identity explorations occur (Arnett, 2000). Through defusion, unpleasant thoughts simply existed as thoughts with no control over one's behaviour (Harris, 2019).

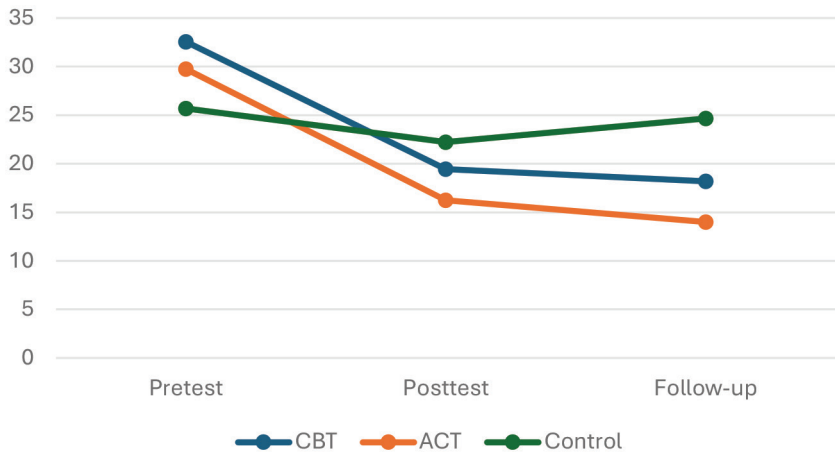

Figure 3.1 Mean for Depression Level for CBT, ACT, and Control Group (Psychoeducation)

Following Kwan et al. (2010), the success of treatment in one intervention over another may be affected by patients' characteristics, such as their preferences or age. As such, having options to select the most appropriate treatment may benefit patients and even therapists. A-Tjak et al. (2018) proposed the application of ACT to prevent depression or its recurrence. Zettle and Rains (1989) similarly concluded that ACT is as effective as cognitive therapy in treating major depression despite their distinct underpinning change mechanisms. Summarily, CBT and ACT are equally effective in reducing depressive symptomatology compared to the control group. Regardless, ACT is superior to CBT in reducing depression among young adults.

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4

ACTion for Depression

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Now, we are going into the gist of the book. The ACT techniques and process inside this book are based on Harris (2019) and Zettle (2007), with an emphasis on participant actions based on their values, regardless of the symptoms. This statement relatively differs from CBT, which primarily focuses on symptom reduction. ACT highlights the six core therapeutic processes involved during the sessions: contacting the present moment, self-as-context, defusion, acceptance, values, and committed action. Essentially, ACTion for Depression constitutes 10 weekly individual sessions ranging from 45 to 60 minutes per session. All the sessions should be conducted by the same counsellor/ mental health professional. The next parts are describing the process and techniques used in formulating the client's case, as well as guiding counsellors in planning the treatment as well as structuring the sessions.

FIRST SESSION AS THE FIRST IMPRESSION

Ever heard of a saying that you never get a second chance to make a first impression? Well, it is not all true but it does make a good point. First impression in counselling or first session is really important. It's not just the counsellor's characteristics and attitudes are important, but also how we introduce the clients to the therapy that we are about to use. In this section, we will focus on the first session to make the ACTion for Depression a good first impression.

ACT is a nonlinear model, thus we can focus on any of the core processes at any time during a session. As a result, there is no set order. However, we aim to do these in the first session; (1) establish rapport, (2) obtain informed consent, (3) history taking, and (4) establish behavioural goals. Plus, with any extra time, we could do a brief experiential exercise and give some simple homework.

Following the rapport building, obtaining informed consent and history taking, you can start to introduce ACT to clients. Explain to them that ACT is an active type of therapy and both of you will collaborate together to help the client in creating the life they desire. Besides, emphasizing on values identification is also crucial in ACT, such as what qualities and strengths the client wants to build, and how they want to treat themselves and others. As the counsellor, you may mention that after each session, there will be a homework, whether it involves writing it down or doing experiential exercise. And you may encourage the client to take risks and explore new experiences that push them beyond their comfort zone, as well as emphasizing on practising at home that will make all the difference.

It seems doable for the first session, right? But bear in mind that client's mind will be sceptical at least for the first couple of sessions. The mind will continue to suggest something like *This isn't going to work*. We can choose to give up and end the session, or we can allow the mind to voice its concern and continue working, putting in our best effort. We're not challenging the client's thoughts; instead, we're acknowledging them as normal and typical. It's okay for the client to have that thought. The thought is present but doesn't control the client's behaviour.

Agreeing on the number of sessions are the next thing you can discuss in the first session. ACT can be delivered in various durations; long-term (40 sessions), brief (12 sessions), and very brief (4 sessions). Obviously, with these short-duration ACT sessions, the client won't fully embrace a mindful, values-driven life, and never encounter problems again. It is more likely we can efficiently and effectively deliver the key aspects of ACT – staying present, open up, and focusing on what matters. Since it isn't the same level of openness to counselling in Malaysia, where I live and work, as there is in the United States or other countries, this book will focus on doing ACT in ten sessions at first, focusing on people with depression symptoms.

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If you have extra time during the first session, you can introduce a brief experiential exercise and a simple exercise that can be done is filling up the Choice Point as it is easy to understand. The Choice Point is among the techniques used in committed action – one of the ACT core processes. The Choice Point is a technique that swiftly outlines issue, pinpoints sources of distress, and develops an ACT strategy to address them. You may use Figure 4.1 to guide you in explaining the Choice Point to the client.

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Basically, towards moves are the things you want to begin or do more of, while away moves are the things you want stop doing or do less frequently. Overt and covert behaviour are included in the choice point. The client characterises what the away move is, not the counsellor. Initially, client might view self-destructive or self-defeating behaviour as a towards moves. As a counsellor, we acknowledge that and jot it down alongside the towards arrow. Over the time, maybe in the next few sessions, the counsellor could address this back.

Depending on the context, any action might be seen as toward move or away move. It's not much about the actions themselves, but rather the effects those actions have on us. As we know, behavioural activation is one of the successful technique in treating depression. Thus, identifying the away moves and towards moves are crucial in depressed people, to guide them view what are their goals and whether the current behaviours are helping them towards achieving their targets or not.

The left arrow in the Figure 4.1 shows when we are getting hooked, happens when our thoughts and feelings hook our attention, derail us and lead us astray. If this happens, we will do the "away moves", which is behaving unlike the person we want to be. Cognitive fusion and experiential avoidance are involved in the hooking skills, in which ACT views these two as the main contributors to our psychological suffering. Cognitive fusion is when we get tightly tangled to our thoughts, up till we get dominated by them. Experiential avoidance is the ongoing attempt to avoid or eliminate the unwanted thoughts and emotions. Experiential avoidance is common in depression, where ACT views affective disorders as the result of individuals' unsuccessful attempts to avoid undesirable private situations that they do not want to encounter.

There are moments when we can detach ourselves from the unwanted thoughts and feelings and focus on towards move, acting in ways that align with the person we want to be. This unhooking skills incorporate all four

mindfulness processes; defusion, acceptance, self-as-context, and being present. All these will be discussed in later section.

Following introduction of the Choice Point, it's easy to incorporate it into basic homework assignments. We can ask the client to notice their towards and away moves. First, pay attention to the towards moves. Among questions that can be asked are "*when do I have them*?" and "*where do I do them*?". Second, pay attention to the away moves and ask them same questions. We may ask the client to fill up the blank Choice Point as their homework.

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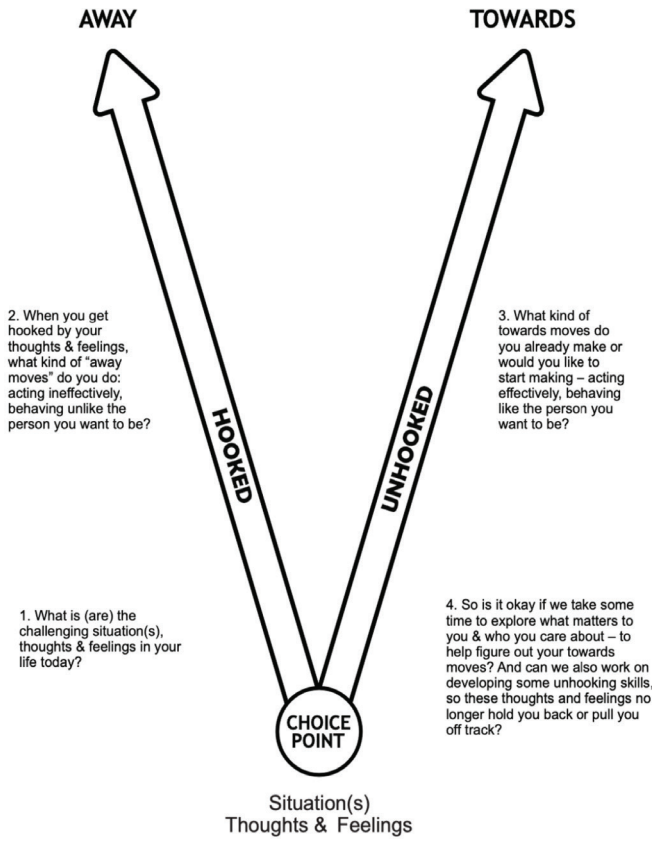

Source: Harris, R. (2019). ACT made simple: An easy-to-read primer on acceptance and commitment therapy. New Harbinger Publications.

Figure 4.1 The Choice Point

TO 2ND SESSION AND BEYOND

Since ACT is a nonlinear therapy, therefore you have multiple choice of steps to follow. There are many suggestions available which step we should go next, depending on the client's needs. However, this book is focusing on people with depression symptoms and most of them are usually focused on wanting to feel good and getting rid of the negative feelings. Therefore, the recommended sequence for the techniques are as stated below. Even though not every people with depression have the same symptoms and issues, we may use the guideline below as our reference only, and not to be too firm with it. Afterall, ACT is all about being flexible with the use of core processes along the sessions.

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Recommended sequence for the techniques/activities:

1. Behavioural goals vs emotional goals
2. Values clarification
3. Creative hopelessness
4. Dropping the struggle
5. Contacting the present moment
6. Defusion
7. Acceptance
8. Self-as-context
9. Committed action

It is being recommended to follow a general structure for every session and we may adapt it accordingly. The first thing to do in the second session and beyond is the practising of mindfulness exercise. It can be beneficial to start each session with a brief mindfulness practice, such as mindful breathing or dropping anchor. The second step to do in every session is reviewing the previous session and homework. Afterwards, we can go into the main agenda of the particular session. The next step is to go into the main intervention. However, it is crucial to be adaptable and respond appropriately to what's happening in the session. If necessary, be prepared to set aside our original plan (we can always return to it at a later time). And every session should end with a homework assignment. The client

should be reminded that their actions between sessions are what can have the most significant impact on their lives.

Behavioural Goals vs Emotional Goals

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Establishing behavioural goals in early therapy is crucial especially in depressed people. Behavioural goals are "doing" goals in the sense that they describe what we intend to do (overt and covert). We may ask the following questions to define overt behavioural goals; (1) *what will you change?*, (2) *what will you begin doing or stop doing?*. We could ask about the following in order to define the covert behavioural goals; (1) *are there any tasks or activities that you'll be able to focus on or engage in more successfully?,* and (2) *with whom will you be paying closer attention or being more present?*.

Talking about client with depression, they often carry their emotional goals into the session, focusing on how they want to feel or they don't want to feel. Sadly, we won't be able to carry out ACT if we agree to those goals. This is so because the ongoing attempt to halt and get rid of unpleasant thoughts and feelings is strengthened by emotional goals in the experiential avoidance agenda. With ACT, our goal is to purposefully diminish experiential avoidance and present an entirely new agenda: experiential acceptance (also known as "willingness").

Usually, the aims of behavioural goals in depression are to; (1) to acquire new skills for managing painful thoughts and feelings – to unhook from them so they don't hinder or drag you down, and (2) to re-engage clients in activities that used to be meaningful to them. Our thoughts and feelings aren't the issue, it's how we get hooked on them (fusion and avoidance) that causes problems. Our behaviour is affected by our thoughts and feelings, but it is not always under their control. So, when do thoughts and emotions have the greatest impact on our behaviour? In an environment of fusion and avoidance, they do. It is easier for us to act in accordance with our values when we are in a defusion and acceptance since the same thoughts and feelings have far less power over our actions (i.e., we unhook from them), enabling us to act on our values. No matter what our thoughts and emotions are, our ability to decide on how we behave is influenced by our psychological flexibility.

Values Clarification

Values are terms that explain how we want to act in the present and ongoing basis. They can be used as a continuous guide for both overt and covert behaviour. Until they have learned defusion and acceptance skills, some clients might not be able or willing to talk about values in great detail. On the other hand, certain clients wouldn't be driven to do therapy tasks until they initially engaged with their values.

However, in clients with depression symptoms, it is useful to clarify the values early in session. They can function as "anchors" for assessing the workability of both behavioural excesses and deficits. Besides, values can act as a guide to plan for committed action interventions. For example, clients who value a good physical health, but has stopped doing exercise because of low motivation following a life crisis, might start doing a minimal exercise inside the house.

Emphasizing on values can help differentiate between situations where clients are unwilling to experience some of the frustration and self-doubt that can occur as a result of obstacles and difficulty, and cases where depression is mainly brought on by the achievement of goals that are opposed to personal values. But how to identify values? These five fundamental ideas regarding values can be used to highlight specific points during therapy; (1) values are present in the moment; goals are for the future; (2) values never require justification; (3) values frequently require prioritization; (4) values are freely selected; and (5) values encompass both oneself and others.

One of the techniques used to identify values are *Ten years from now, looking back*. Ask the client to imagine themselves in the future, ten years from now, and they're looking back on their current life. Complete the following statements; (a) *I worried too much about …,* (b) *I didn't spend adequate amounts of time doing things like …,* and (c) *if I were to go back in time, I would do things differently like …* . These questions can help guide the client in clarifying their values. After clarifying values, you may ask the client to do these as their homework; (1) notice when they're acting on their values, and (2) pay attention to how it feels to do so and what difference it creates. If we are able to elicit core values in the session, the following session can be focused on values-based problem solving and action planning. If one has obstacles to implementing their values, such as fusion and experiencing avoidance, we would likely focus on defusion and

acceptance as a solution. This is especially likely to occur in those who are depressed. Due to that, next section will be on dropping the struggle.

Creative Hopelessness

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Creative hopelessness (CH) is a state where one realizes that attempting to stop or eliminate undesirable thoughts and emotions makes things worse rather than better. The aim is to consciously recognize and admit that sticking to this goal is unworkable. When a client is overly experientially avoidant and firmly committed to controlling their emotion, this technique is used. It can also be used when client is refusing or unable to clarify their values during the first session. We can proceed with CH, later going back to values clarification. We refer to it as "creative hopelessness" since our goal is to instil a sense of hopelessness to manage our emotions; this is certainly not about losing hope for the future, our lives, or ourselves.

This technique is not a necessary for everyone. We can skip CH if the client is motivated to get better and isn't overly invested in an emotional control agenda, or if they are open to and already feel at ease with mindfulness or ACT. As a counsellor, you can ask the client five simple questions; (1) *what did you try before?,* (2) *how did it turn out?,* (3) *what has it cost?,* (4) *what emotions are surfacing?,* and (5) *do you have an open mind to something new?*.

Dropping the Struggle

Following CH, the client may ask, *"so what should I do?"*. The counsellor could help them to view that the problem does not lie in the unpleasant thoughts or feelings per se, but rather in the way we resist against them. Pushing away paper metaphor is best to describe this. Take a sheet of paper and try to imagine that in front of you are all things that matter to you, as well as all the difficulties and obstacles you're facing in your life. Next, have the client imagine that the piece of paper is a representation of every thoughts, memories, or feelings they would like to let go of. Push the paper as far away from you as you can while holding it firmly at the edges. Ask the client three things; (1) how exhausting it is, (2) how distracting it is, and (3) how hard it is to take action and accomplish things in our lives. Next, you can provide a brief homework to your client that expands on the

metaphor of struggle they have used. Ask them to identify times in which they are "pushing hard" and the effects that will occur if they stop pushing.

Contacting the Present Moment

SCOPUS

Dropping anchor is an easy but effective technique for interacting with the present moment. Its purpose is to assist clients in being "completely present," engaging in what they're doing, regaining control of their behaviours, and concentrating their attention on what matters most right now. To introduce dropping anchor to clients, we can explain to them about the emotional storm metaphor (Harris, 2019). It can be described as an emotional storm is building inside you and there's nothing you can do when you're being blown away by the storm. Dropping anchor should be our first step of action when we're experiencing an emotional storm. It goes without saying that this won't make the storm go away because anchors have no control over the weather, but it will keep us stable until the storm passes.

You may use your own script to do the Dropping Anchor Exercise and you may watch some video as a reference to the exercise. Before you start this activity, the client should have provided us with information on the thoughts, feelings, and memories that are part of their emotional storm. If that's the case, we should refer to them specifically. We may use general terms like "pain," "discomfort," "hurt," "difficult thoughts and feelings," or even "emotional storm" to describe the client's thoughts and feelings if they are unable to express or describe them. As you perform the exercise, use a gentle and soothing tone of voice and demonstrate how to perform each movement (e.g. pressing feet down, moving the hands, etc.). The counsellor continues the exercise by recognizing inner experience, coming back to the body, and engaging with the outside environment until the client is anchored, which can be shown by nonverbal cues or vocal cues indicating engagement.

There are several phrases that you can use in the script while doing the exercise, such as; "*Note your painful thoughts and emotions.*" This is the primary (and crucial) difference that sets ACT's grounding exercise apart from other models. These exercises are unlikely to be mindfulness exercises and only act as a diversion if you don't often acknowledge the presence of the discomfort and pain on a regular basis. You can end the

technique by asking, *"Do you feel any difference now? Are you less hooked by these disturbing thoughts and feelings? Do you have more charge over your actions now?"*

As a homework assignment, have clients practice dropping anchor whenever and whenever—even at pleasant emotional weather. Your boat will start drifting out of the harbour if you don't anchor it, even on a hot and bright day. Without regular practice, they won't be able to perform it when they really need to.

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Fusion

Before talking about defusion, we might want to take a look at what is fusion. When our thoughts take control of our behaviour—either overt or covert—in a way that is counterproductive or disruptive, it's referred to as cognitive fusion, or fusion. In simpler terms, fusion causes our thoughts to negatively influence our behaviour. We use the term 'fusion' to describe when our overt or covert behaviour becomes confined, rigid, and inflexible to a degree that is counterproductive and self-destructive (e.g., deterioration of health and well-being, pulls us away from our values, makes life miserable in the long run). Clients with depression frequently engage in escape behaviours such as social disengagement, loss of interest, poor energy, and self-focused negative attention, where cognitive fusion is the major component of these behaviours. In cognitive fusion, clients' tend not to see thoughts as merely thoughts, but as actual true events. As a result, they will have the goals of experiential avoidance which is described as distraction and avoidance from the unwanted thoughts, emotions, and painful memories. The thought's content is rarely considered troublesome; the problem is caused by the fusion and avoidance with the thought and when we respond to the thought in fixed, inflexible ways.

Experiential Avoidance

Experiential avoidance is our desire to keep away from or discard any unwanted or unwelcome "private experiences". We are more likely to suffer mentally over time if we spend a lot of time and effort attempting to avoid or get rid of unpleasant personal experiences. It is normal to have experiential avoidance, it is neither "evil" nor "pathological" by nature.

To the degree that it gets in the way of a rich and meaningful life, we just target it when it is unnecessary, rigid, or unacceptable.

Nonetheless, ACT does not advocate for the unconditional acceptance of all thoughts and emotions. ACT promotes experiential acceptance in two scenarios; (1) when there is minimal or impossible avoidance of thoughts and feelings, and (2) when it is possible to avoid feelings and ideas, yet doing so eventually makes life more difficult. Fusion produces an excessive experiential avoidance with this rule: *"Such thoughts and emotions are evil, so I need to get rid of them."* This overlap between processes is why we refer to both fusion and avoidance by using the word "hooked".

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Defusion

Fusion refers to when our thoughts take control over our actions. Defusion refers to the ability to react flexibly to our thoughts so that they can affect but not control our actions. From the very first session, we cultivate the foundations of defusion by simply asking basic questions during history taking. Concepts like "listening in" to the thoughts and observing what "it is doing" can help with defusion. Defusion does not have a fixed sequence or steps. However, we can use 3 Ns of Defusion to guide us.

The first Ns of Defusion is *Notice*, that is to notice what cognitions are present. You can say things like; *"notice what you're thinking." or "notice what your mind is doing."*. The next Ns is to *Name,* which is naming the cognition or cognitive process. The last Ns is to *Neutralize,* that is placing our thoughts in a new context that reduces their power and disarms them. Any one or more of the following can be used to neutralize; (a) treat the thoughts like an object and observe it, (b) describe the thought, (c) play with the properties of the thought (visual, auditory, kinaesthetic), and (d) give the cognitive process a character.

Acceptance

Acceptance means opening up and allowing our inner experiences (thoughts, images, memories, feelings, emotions, desires, impulses, and sensations) to be as they are, regardless of how pleasant or painful they may be. The first step towards acceptance or willingness is to stop battling

and escaping our own emotions and thoughts. We encourage acceptance if and when experiential avoidance is hindering effective values-based living. The client will be trapped in an endless circle of misery as long as they are hooked on the goal of controlling their emotions. To proceed acceptance with client, the counsellor has to make sure that the client has understand and accept the unworkability of their experiential escape behaviours. Otherwise, they need to proceed or redo creative hopelessness.

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The three As of Acceptance are comprised of *Acknowledge, Allow* and *Accommodate*. Acknowledge is define as observing the inner experience with curiosity and naming it in a nonjudgmental way. Subsequently, we give it permission to remain and to be left on its own; "*I don't want this feeling, but I'm going to let it be*". Next is to accommodate, that is to give them space, allow them to stay for however long they like, and adjust to having them stay with us. There are many exercises to encourage acceptance. The following are some exercises you can do with client and example of scripts to guide you. You may choose one or two exercises during the initial introduction of acceptance to client.

1) The part that notices

Counsellor: So there's a part of you that notices everything, always there, always noticing. And in this exercise, you're going to use that part of you to step back and observe your difficult feelings, without getting pulled into them or swept away by them.

2) Radio mind

Counsellor Let your mind chatter away like a radio playing in the background ... and keep your attention on the feeling ... And at any point, if your thoughts hook you and pull you out of the exercise, the moment you realize it, acknowledge it, unhook, and refocus ...

3) Notice

Counsellor: If you drew an outline around it, what shape would it have? ... Is it 2D or 3D? Is it on the surface of the body or inside you, or both? ... How far inside you does it go? ... Where is it most intense? ... Where is it weakest?

4) Name

Counsellor: Take a moment to name this feeling ... What would you call it? ... Okay, silently say to yourself, I'm noticing a feeling of X ...

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5) Allow

Counsellor: And see if you can just allow this feeling to be there. You don't have to like it or want it ... Just allow it ... Just let it be ... Observe it, breathe into it, open up around it, and allow it to be as it is. (Pause 10 seconds.) You may feel a strong urge to fight with it or push it away. If so, just acknowledge the urge is there without acting on it. And continue observing the sensation. (Pause 5 seconds.) Changing or getting rid of it is not the goal. Your aim is simply to allow it ... to let it be.

6) Physicalize (Objectify)

Counsellor: Imagine this feeling is a physical object ... As an object, what shape does it have? ... Is it liquid, solid, or gaseous? If you could touch the surface, what would it feel like? ... Wet or dry? ... Rough or smooth? Observe this object curiously, breathe into it, and open up around it ... You don't have to like it or want it. Just allow it ... and notice that you are bigger than this object ... no matter how big it gets, it can never get bigger than you.

7) Be Self-compassionate

Counsellor: Simply put your hand where you feel most intense—and feel the warmth flowing from your hand into your body ... not to get rid of the feeling but to make room for it ...

8) Expand Awareness

Counsellor: Life is like a stage show ... and on that stage are all your thoughts, and all your feelings, and everything that you can see, hear, touch, taste, and smell ... And what we've been doing

here is dimming the lights on the stage, and shining a spotlight on this feeling ... and now it's time to bring up the rest of the lights ... So keep this feeling in the spotlight, and at the same time, bring up the lights on your body ... notice your arms and legs and head and neck ... and notice that you're in control of your arms and legs, regardless of what you're feeling ... Just move them around a little to check that out for yourself ... and now take a stretch, and notice yourself stretching ... And also bring up the lights on the room around you ... Open your eyes, look around, and notice what you can see ... and notice what you can hear ... and notice that there's not just a feeling here; there's a feeling inside a body, inside a room, where you and I are working together on something very important ... and welcome back!

SCOPUS

Clients frequently say that an unpleasant feeling or sensation reduces and frequently disappears when they accept it. It is important to note that this is a bonus and not a target, and it doesn't happen every time, so don't count on it when it does.

Self-as-context

Self-as-context (SAC) is the part of us that does all the noticing. Metaphorically, we seem to possess a "safe place" inside of us where we are able to "open up" and "make space" for difficult thoughts and feelings. It's similar to being able to "step back" and watch your thoughts and feelings from a "perspective" or "viewpoint." In other words, SAC is "the observing self" or "the part of you that notices".

Everything continues to be seen from an I/here/now locus or perspective, which is the constant locus or perspective from which this noticing takes place. SAC is what ACT refers to as the constant locus from which we notice everything. Request to the client to spend some time exploring SAC. If the client agrees, we'll normally start by introducing a metaphor to help the client understand SAC, and then a mindfulness exercise to help them actually experience it. A simple yet powerful metaphor to be used is The Stage Show Metaphor.

The Stage Show Metaphor:

"I think it's sometimes helpful to view life as if it were a huge stage show. And since it's such a huge stage, there's a lot going on. You have all of your thoughts and feelings, as well as everything you can see, hear, touch, taste, and smell, as well as all of the physical activities you engage in with your body. And the show is always shifting, from one instant to the next. And, you know, what is up there on the stage can be great at times, but it can also be pretty bad.

There's a part of you that can take a step back and enjoy the performance. Always aware, always watching. You can zoom in on any part of the show to get a closer look at the details, or zoom out to see the larger picture.

The show is always shifting, from one moment to the next. However, the part of you that watches it remains unchanged. It's always there, always paying attention, no matter what happens on stage, no matter how good or bad the show is."

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This metaphor relates to earlier work on defusion and acceptance: thoughts and emotions are like actors seeking for your undivided attention; if you let them, they'll take hold of you and pull you up onto the stage (fusion). Following the metaphor, you can perform the following mindfulness exercise with client. And once the noticing self has been introduced, we can use it as a quick intervention to help with acceptance and defusion throughout sessions.

Counsellor:

Find a comfortable position, and close your eyes. Now notice: where are your thoughts? ... Where do they seem to be located: above you, behind you, in front of you, to one side? (Pause 10 seconds.) And notice the form of those thoughts: are they pictures, words, or sounds? (Pause 10 seconds.) And notice—are they moving or

still? ... And if moving, what speed and what direction? (Pause 10 seconds.) Notice there are two separate processes going on here: there's a process of thinking—your mind is throwing up all sorts of words and pictures—and there's a process of noticing—there's a part of you noticing all those thoughts. (Pause 10 seconds.)

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Now this gets your mind whirring, debating, and analysing, so let's do it again.

Notice where your thoughts are located ... Are they pictures or words; moving or still? (Pause 10 seconds.) So there go your thoughts—and there's a part of you able to notice them coming and going. Your thoughts keep changing. But the "you" that notices them does not change.

Now once again, this gets your mind whirring, debating, and analysing, so let's just do that one last time. Notice: where are your thoughts? ... Are they pictures or words; moving or still? ... (Pause 10 seconds.) There go your thoughts—and there "you" are, observing them. Your thoughts change; the you that notices, does not.

Mindfulness Skills

The core mindfulness skills - defusion, acceptance, contacting the present moment, and self- as-context can be learned in a variety of ways, including formal meditation. In other words, mindfulness doesn't necessarily means meditation. Rather than getting sucked into or clinging to thoughts, the mindfulness exercise teaches us to step back and notice them as they come and go.

The Leaves on a Stream exercise involves visualizing a softly flowing stream with leaves on the water's surface, then placing our thoughts on the leaves and allowing them to float away. The purpose of the practice is not to make our thoughts vanish, but to learn how to step back and look at the movement of our thoughts. Approximately 10% of individuals find

visualization to be very challenging; in this situation, we need to provide a nonvisual choice. One good option is to close our eyes and focus on the dark area behind our eyelids rather than attempting to see a stream and leaves. This is a technique that we can practice outside of stressful conditions to improve our unhooking skills. In situations like this, it's best to unhook quickly, ground ourselves, and concentrate on the task at hand. Following is the example script for The Leaves on a Stream Exercise.

SCOPUS

1. Find a comfortable position, and either close your eyes or fix your eyes on a spot.
2. Imagine you're sitting by the side of a gently flowing stream, and there are leaves flowing past on the surface of the stream. Imagine it however you like—it's your imagination. (Pause 10 seconds.)
3. Now, for the next few minutes, notice each of your thoughts as it pops into your head ... then place it onto a leaf, and allow it to come and stay and go in its own good time ... Don't try to make it float away, just notice what it does ... It may float on by quickly, or it may go very slowly, or it may hang around ... Do this regardless of whether the thoughts are positive or negative, pleasurable or painful ... even if they're the most wonderful thoughts, place them onto a leaf ... and allow them to come and stay and go, in their own good time ... they may float by quickly, or slowly, or they may hang around ... simply notice what happens, without trying to alter it. (Pause 10 seconds.)
4. If your thoughts stop, just watch the stream. Sooner or later, your thoughts will start up again. (Pause 20 seconds.)
5. Allow the stream to flow at its own rate. Don't speed it up. You're not trying to wash the leaves away—you're allowing them to come and go in their own good time. (Pause 20 seconds.)
6. If your mind says, This is stupid or I can't do it, place those thoughts on a leaf. (Pause 20 seconds.)
7. If a leaf gets stuck, let it hang around. Don't force it to float away. (Pause 20 seconds.)
8. (An optional add-in: introducing acceptance) If a difficult feeling arises, such as boredom or impatience, simply acknowledge it. Say to yourself, Here's a feeling of boredom or Here's a feeling of impatience. Then place those words onto a leaf.

9. From time to time, your thoughts will hook you, and pull you out of the exercise, so you lose track of what you are doing. This is normal and natural, and it will keep happening. As soon as you realize it's happened, gently acknowledge it and then start the exercise again.

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Keep going with the exercise for a few minutes after instruction 9, adding this reminder every now and then: "*Again and again, your thoughts will hook you. This is normal. As soon as you realize it, start up the exercise again*." We can conclude the practice by dropping anchor again or by giving a straightforward directive like: "*And now, bring the exercise to an end ... and sit up in your chair ... and open your eyes. Look around the room ... and notice what you can see and hear ... and take a stretch. Welcome back*!"

In any mindfulness activity, we can emphasize defusion by (a) adding statements such; *"Notice how your mind hooks you"* or *"As soon as you realize you've been hooked, acknowledge it, unhook yourself, and refocus"* and (b) adding metaphors about *"letting your thoughts come and stay and go".* Apart from the above metaphor, we may use the following metaphors to allow our thoughts to come and go as they please; (a) vehicles passing by outside your home; (b) clouds floating in the sky; (C) pedestrians crossing the street; and (d) gentle waves crashing into the shore.

One may ask, how long do meditative exercises go for? It all comes down to (a) meeting the situation's needs and (b) taking the client's abilities into account. We can begin with a four- to five-minute exercise as a test and modify it in response to the client's feedback. You can shorten any of ACT's meditation exercises to three to four minutes, or you can extend them to twenty or thirty minutes. If the thoughts are difficult for our clients to "visualize" or "see," focus on auditory characteristics such as pitch, pace, tone, and emotionality, and observe the thoughts as though we were listening to someone.

Committed Action

Depression's numbing effect as a form of experiential avoidance has a negative impact on motivation that hinders positive behaviours. Due to this reason, most behavioural activation treatments are successful in treating depression. In ACT, behavioural activation is the component in committed

action, where it is defined when client is taking meaningful action that is directed and inspired by values. This encompasses both physical (overt) actions and psychological (covert) actions. Committed actions include in-sessions activities and after sessions homework. It is also beneficial for us to identify client's previous pleasurable or enjoyable activities. This information can aid us to propose an appropriate action plans and act towards it.

Solving problems, defining goals, and action planning are the three most frequently taught skills in ACT. ACT uses the same basics in problem solving, except that ACT preceded these steps with mindfulness and values. We won't jump right into problem solving if client is fused, struggling, and frustrated by emotions; instead, we'll assist them unhook from the unwanted thoughts and dropping anchor. If client is reluctant to proceed with the committed action, we can present to them three options: (1) leave; (2) stay and live up to their values, which require them to make all the necessary adjustments to better the situation and accept the accompanying pain; or (3) stay and stop acting effectively, which entails doing things that are pointless or could make the situation worse.

Option 1 isn't always available. However, if leaving the situation is a viable choice, do so. Is leaving a high-conflict relationship or a meaningless job going to make your life better, fuller, and more purposeful than staying? Option 2 includes making room which consists of dropping anchor, defusion, acceptance, and self- compassion. And option 3 comes instinctively to all of us; in tough circumstances, difficult thoughts and emotions can quickly hook us and take us away from our goals.

For goal setting, we can use the SMART goal acronym; S : Specific, M : Motivated by values, A : Achievable, R : Realistic, and T : Time-framed. To ensure that a goal is effective, we must ask the question, "*Is it a live person's goal?".* A dead person's goal is one that represents what you would not do. For example, client might say that "*This week, I'm not going to scream at my kids.*" – this is an example of a dead person's goal as the dead one cannot scream at people. We have to rephrase that goal into a live person's goal, for example, "*This week, when the kids throw tantrum, I'm going to drop anchor, breathe into my anger, connect with my values of composure and patience, and talk to them calmly".*

But what happen if the client's goal is to change others? We have to remind them that they can only influence other people, not control them.

The client has the ability to control their own actions, and the better they do so, the more they will be able to influence others. There will be moments when they are unable to achieve what they desires; how does they plan to handle these situations? What kind of towards moves will they make if that happens? What are the values they want to live by (a) if they succeed and (b) if they fail?

At times, clients will have impossible goals. When a goal seems unlikely or far away, here's what we can do; (a) validate the suffering caused by the large "reality gap.", (b) acceptance, defusion, and self-compassion are effective ways to deal with pain, (c) find the value(s) underlying the goal, (d) create new goals that are focused on the underlying value(s) that are achievable for the current situation. Learning to take small steps is important. Living our values is a never-ending journey that will last until our last breath. And every small step we take, no matter how small, is an important and valid part of the journey. Besides that, we can assist clients to divide goals into three categories: immediate, short-term, and long-term.

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5

ACTion Overload and other Pitfalls

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We may choose two key techniques for each of the six core processes—tools, worksheets, activities, metaphors, practices, questions, and so on. This will provide us with a core set of twelve interventions that we can mix and match, adapt, and co-opt for a variety of issues. Below is the example of processes and techniques available. See if we can restrict our practice to experimenting with these twelve interventions for at least a few weeks, before they are so familiar to us that they come easily and efficiently.

Table 5.1 Example of techniques/exercises available for ACT core processes

Core Process	Techniques	
Values	Ten Years from Now, Looking Back	The Bull's Eyes
Contacting the present moment	The Emotional Storm Metaphor	Dropping Anchor
Defusion	Bad Chair Metaphor	Hands as Thoughts and Feelings
Acceptance	Physicalize	Expand Awareness
Self-as-context	The Stage Show Metaphor	There Go Your Thoughts
Committed action	The Choice Point	Towards Moves

There are some practical tips in doing experiential exercise with your client. Slow down more if you need to. If you're uncertain, ask your client, *"am I going too fast or too slow for you, or am I about right?"*. And if you're new to ACT, play around with these techniques. Try to adapt them accordingly and change the words, the imagery, and the objects in the scene. We may combine elements of one exercise or technique with others. End mindfulness exercises with a bit of dropping anchor or a quick link to values is also beneficial. After the experiential exercises are completed, it is necessary to debrief them – *"how might this help you with your issue/ goal?"*. And lastly, turn the exercise into action plan/experiment.

When a client says, "*it's not working,"* we should always answer with openness and curiosity (which typically means dropping anchor, making space for our own anxiety, and unhooking from our own unhelpful thoughts). We have to make it clear to them that the purpose of doing those exercises are not to suppress the unwanted thoughts and emotions, nor regulating how we feel or making we feel better. We have to recap some metaphors (such as pushing away paper metaphor) to emphasize on this.

Keep calm when something in a therapy session fails, goes wrong, or backfires. Of course, we won't be at ease. We should, however, behave calmly. Detach from our fears of messing it up, make room for our anxiety, drop anchor, and be completely present. These are the steps in reacting to the client's negative reactions. First, counsellors should defuse from their own fears and self-judgment, acknowledges their anxiety and frustration, grounds themselves, and cultivates an attitude of openness and curiosity. When the client seems to be fused, the counsellor switches the process from contacting the present moment (dropping anchor) to defusion (noticing and naming the cognitions). Next is to acknowledge the client's negative reaction as normal and expecting it will continue. And then inquire whether there was something we said or did that provoked this reaction in the client.

Other pitfalls of any therapy is relapse. When a desired behaviour change has been implemented and a number of goals have been met, it is time to either stop or reduce the frequency of therapy. The goal of ACT is to help client to get unstuck. It's not about reaching the end of one's life. The counsellor must ensure that the client is open to behaviour change before termination to minimize the likelihood of relapse. For this, ask the client to rate from 1 to 10 scale to measure how open they are to carry out the

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homework. One represents no commitment at all, and 10 represents total devotion. At this time in therapy, client should be able to provide a rating of 7 or higher. If the score is less than 7, counsellor should discuss with clients the reasons behind this and may consider extending the number of session.

It is important to prepare for setbacks after termination. During the last two sessions, counsellor could ask, (1) ask client to imagine what will they do if they start to get hooked again by the unwanted thoughts, feelings, or memories?, (2) counsellor may help the client respond to this by writing down the techniques learnt which are beneficial for clients. Encourage clients to write down self-therapy notes and keep it safely. This can be very helpful, especially at times where the client encounters hardship after termination, and they can refer to the notes back. And apart from the self-therapy notes, reassure clients that they can always seek for professional helps whenever they have setbacks. Following is the example of self-therapy notes.

Self-therapy notes:

1. What are my unwanted thoughts, feelings or memories?
2. Am I hooked on them?
3. Fill up the Choice Point.
4. Read back on Creative Hopelessness.
5. Am I acting on my values?
6. Carry out The Dropping Anchor exercise.
7. What escape behaviours do I exhibit?
8. "There Go My Thoughts..."
9. Carry out the Leaves on a Stream Exercise.
10. Do I have an achievable action planning?
11. What are the payoffs and costs of my behaviour?

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6

Brief ACTion Planning

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A treatment planning is crucial in counselling session in order for client to fully benefit from it, as well as helping counsellor to plan ahead. Table 6.1 shows the brief treatment planning for ACTion for depression in 10 individual sessions. Again to be reminded that there is no fixed order in ACT. We may use the planning below as the guideline only and not to be too firm with it. Afterall, ACT is all about being flexible with the use of core processes along the sessions. You may be able to finish all the planning in the Table 6.1 within the 10 sessions, but let's face the reality, not all the things we plan will be a success. There will always going to be lumps and bumps along the journey. Be flexible. For example, if you aren't able to finish it within the 10 sessions, then extend it a little bit longer.

Table 6.1 Brief treatment planning for ACTion for Depression

Session	Process, techniques, and interventions	Core process involved
1st	Introduction to ACT. Identifying presenting problems. Obtaining relevant past history. Establish behavioural goals and their relationship to values. Introduction on Choice Point (**Committed Action**) Setting up homework: Fill up the blank Choice Point. Noticing Towards and Away Moves on Choice Point. And other relevant plan Summary. Client's feedback.	The Choice Point (**Committed Action**) is introduced early in session to guide client identify their behavioural goals and **Values**.
2nd	Review of homework. Setting up agenda. Discussion of issues on the agenda. Main interventions: **Creative Hopelessness**: Person in a hole metaphor. **Values** clarification: Ten year from now, looking back. Setting up homework: Notice when you're acting on your values and pay attention to how it feels to do so. Review back on creative hopelessness. And other relevant plan 6. Summary. 7. Client's feedback.	Initially, proceed with **Values** clarification. Only if clients are too focused on eliminating the unwanted thoughts/ feelings, then we proceed with **Creative Hopelessness** first.

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Table 6.1 *cont'd*

Session	Process, techniques, and interventions	Core process involved
3rd	Review of homework. Setting up agenda. Discussion of issues on the agenda. Main interventions: **Drop The Struggle**: Pushing Away Paper Metaphor. Introduction on **Contacting The Present Moment:** The Emotional Storm Metaphor. The Dropping Anchor exercise (**Contacting the Present Moment**). Setting up homework: Notice when you're "pushing hard" and what difference it will make when you stop pushing. Practise Dropping Anchor. And other relevant plan. Summary. Client's feedback.	**Drop the Struggle** is best done following **Creative Hopelessness**. After the client has understand that their struggle against difficult thoughts are the problem, not the thoughts themselves, only then we introduce **Contacting The Present Moment**
4th	Review of homework. Setting up agenda. Brief mindfulness exercise: The Dropping Anchor exercise **(Contacting the Present Moment).** Discussion of issues on the agenda. Main interventions: Introduction on cognitive **fusion**: *What's hooking you?* Identify client's experiential avoidance/ escape behaviour. Establish connection between **fusion** and experiential avoidance. Setting up homework: Keep practising Dropping Anchor. Write down any other cognitive fusion and experiential avoidance that happened before. And other relevant plan. Summary. Client's feedback.	Shows client how **fusion** produces an excessive experiential avoidance – "*I should not have these thoughts, I need to get rid of them*" – which hooks them, and leading to away moves.

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Table 6.1 *cont'd*

Session	Process, techniques, and interventions	Core process involved
5th	Review of homework. Brief mindfulness exercise: The Dropping Anchor exercise **(Contacting the Present Moment).** Setting up agenda. Discussion of issues on the agenda. Main interventions: Introduce **Defusion:** Bad Chair Metaphor. Deeper into **Defusion:** Hands as Thoughts and Feelings. Setting up homework: Practise Hands as Thoughts and Feelings for any other negative thoughts and feelings. Practise Dropping Anchor And other relevant plan. Summary. Client's feedback.	Following **Values** clarification and **Contacting The Present Moment, Defusion** is introduced where experiential exercise is done to help them defuse from unwanted private events. Dropping anchor is still done consistently as brief mindfulness exercise and action plan.
6th	Review of homework Brief mindfulness exercise: The Dropping Anchor exercise **(Contacting the Present Moment).** Setting up agenda. Discussion of issues on the agenda. Main interventions: Check whether client has understand and accept the unworkability of her experiential escape behaviours. Introduce **Acceptance:** Physicalize the painful feelings. Introduce **Self-as-context:** The Stage Show Metaphor. Exercise for **Self-as-context**: There Go Your Thoughts... Setting up homework: Practise techniques in Acceptance Practise brief mindfulness exercise (Dropping anchor or There go your thoughts). And other relevant plan Summary. Client's feedback.	**Acceptance** involved Acknowledge, Allow, and Accommodate. **Self-as-context** is the part of you that notices. The Stage Show Metaphor can be linked to **defusion** and **acceptance:** thoughts and emotions are like performers in a show **(acceptance),** trying their best to catch your full attention – and if you're not careful, they'll grab hold of you (**fusion**).

Table 6.1 *cont'd*

Session	Process, techniques, and interventions	Core process involved
7th	Review of homework. Brief mindfulness exercise: The Dropping Anchor exercise **(Contacting the Present Moment).** Setting up agenda. Discussion of issues on the agenda. Main interventions: Combine the 4 core processes (**defusion, acceptance, contacting the present moment, and self-as-context**) into one exercise: The Leaves on a Stream Exercise. Explain the rationale of this mindfulness exercise in daily life. Setting up homework: Practise The Leaves on a Stream Exercise. Notice when you're hooked during the practise and what do you do to handle it. And other relevant plan. Summary. Client's feedback.	After learning the core mindfulness sills – **contacting the present moment, defusion acceptance,** and **self-as-context**, we can clump them into one exercise.
8th	Review of homework. Brief mindfulness exercise: The Leaves on a Stream Exercise. Setting up agenda. Discussion of issues on the agenda. Main interventions: Problem solving, goal setting, and action planning (**Committed Action**). Setting up other SMART behavioral goals. Setting up homework: Incorporate mindfulness exercise into performing committed action. And other relevant plan. Summary. Client's feedback.	**Committed action** is also introduced on early session (The Choice Point). This session is deeper into the process and focusing more on taking action while incorporating mindfulness exercise.

Sample

Table 6.1 *cont'd*

Session	Process, techniques, and interventions	Core process involved
9th	Review of homework. Brief mindfulness exercise: The Leaves on a Stream Exercise. Setting up agenda. Discussion of issues on the agenda. Main interventions: Using Choice Point to: a) Validate payoffs and highlight costs of client's unworkable behaviour. b) Highlight payoffs and validate costs of client's workable behaviour . Preparing client for termination. Setting up homework: Continue doing the workable behaviour, guided by values, and incorporating mindfulness skills into the action. Identifying concerns on termination. Summary. Client's feedback.	We need to recognize and validate the payoffs (outcomes the client wants) and compare them with the costs (outcomes the client does not want) to see if the behaviour is workable – this can help in executing the **Committed action.**
10th	Review of homework. Brief mindfulness exercise: The Leaves on a Stream Exercise. Setting up agenda. Discussion of issues on the agenda Process Reactions to Termination. Summarize self-therapy notes: List out the techniques learnt that are beneficial for client. Summary. Client's feedback. Session termination.	Address any concern client may have after termination. Together, list down the techniques learnt which client can carry out after termination.

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7
Wrap-up

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ACT seems to be as effective as CBT in reducing depression symptoms, especially among young adults aged 18 to 29 in Malaysia. Although ACT is a newer therapy and claimed to be more suitable for more severe and treatment-resistant patients (Kenny & Williams, 2007), Ahmad Othman et al. (2024) demonstrated that ACT is more effective than CBT in reducing young adults' depressive symptomatology. Resultantly, acceptance-based therapy techniques may be more effective in facilitating young adults to manage the overwhelming thoughts and feelings accompanying depression symptoms. Rather than instructing clients to label and dispute their distressing thoughts, it may be more advantageous to approach the situation with an accepting and non-judgmental mind.

Depression symptoms hindered personal growth and interrupted identity development among young adults (Kuwabara et al., 2007). Following the rising number of depressed young adults (Institute for Public Health, 2020) and those who are susceptible to depression symptoms (Boyne & Hamza, 2022), this book may contribute to guidance and counselling with an emphasis on ACT among young adults. It would be beneficial to incorporate ACT in counselling session among young adults in Malaysia. Eastern cultures advocated acceptance compared to their Western counterparts, which focuses more on controlling the depression-related stressors (Reangsing et al., 2022). This book proposed ACT as among the best therapy to treat young adults with depression symptoms, other than CBT.

ACT proved to be more effective compared to CBT among young adults in Malaysia. This age group may benefit more from ACT, specifically in terms of improving depression, anxiety, and physical and psychological health. The ACT participants are taught on cognitive fusion, which occurs when individuals' cognition controls their behaviour in a disruptive and self-defeating manner. Similar to CBT, cognition or cognitive distortion may adversely impact personal behaviour if not controlled. The CBT participants are taught to dispute and address their thoughts. Contrary to ACT, participants are taught not to eliminate or address personal thoughts but encouraged to exercise psychological flexibility: learning to withdraw from thoughts and allow these thoughts without having to challenge or address them. Despite the variations of the targets and mechanisms underlying CBT and ACT, there is overlap in how thoughts impact one's life. It is primarily people's views and reactions to these thoughts that differ.

In line with past works, young adults (mainly lower-income females) are psychologically affected by the Covid-19 pandemic (Thorpe & Gutman, 2022). Psychological distress involving depression impacts their mental health, which is associated with reduced physical activity (Hanani et al. 2022) and quality of life. ACT is shown to be effective in improving depressed people's quality of life since it focuses on enhancing functioning in several valued domains rather than symptom reduction. Overall, ACT could provide a more comprehensive approach to behaviour modification and improved quality of life.

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Biodata of Authors

Dr. Akmarina Ahmad Othman is a PhD holder in Guidance and Counselling from Universiti Putra Malaysia. She is a senior lecturer at the Department of Counsellor Education and Counselling Psychology, Faculty of Educational Studies, Universiti Putra Malaysia, as well as a registered counsellor of Board of Counsellors Malaysia. Her specializations are in mental health counselling and guidance and counselling.

Assoc. Prof. Dr. Wan Marzuki Wan Jaafar has a PhD in Guidance and Counselling from Universiti Putra Malaysia. He is the Dean and Associate Professor at the Department of Counsellor Education and Counselling Psychology, Faculty of Educational Studies, Universiti Putra Malaysia. His field of reseach are mainly on career development and counselling, guidance and counselling, and counsellor education and supervision.

Assoc. Prof. Dr. Zaida Nor Zainudin is a PhD holder in Guidance and Counselling from Universiti Putra Malaysia. She is Associate Professor at the Department of Counsellor Education and Counselling Psychology, Faculty of Educational Studies, Universiti Putra Malaysia. Her specializations are in e-counselling, guidance and counselling, and counselling education.

Dr. Yusni Mohamad Yusop obtained her PhD in Counselling from Universiti Malaya. She is a senior lecturer at the Department of Counsellor Education and Counselling Psychology, Faculty of Educational Studies, Universiti Putra Malaysia. Her field of interests are guidance and counselling, module development, and family counselling.

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Index

acceptance 3, 6-9, 13, 15, 17-18, 20, 23-27, 30-31, 34-36, 38-40, 46-47, 49, 51-59
  and Commitment Therapy 3
ACT 3-9, 12-25, 27-28, 30, 33, 37-38, 40-41, 43-44, 49-50, 54, 56-59

behaviour 5-6, 14, 18, 21-22, 25-26, 29, 50-51

committed action 3, 6, 8-9, 20, 22, 24, 26, 38, 40, 47-48
counselling 20-21, 43, 49, 61
counsellor 20-22, 27-28, 31, 41-43, 61
creative hopelessness 24, 27

defusion 3, 6, 8-9, 17-18, 20, 23-26, 29-30, 34-35, 37-41, 46-47
depression 1-5, 1-19, 21-22, 24-26, 29, 37, 43, 49-59

experiential avoidance 22, 29, 55

hook 22, 31, 37-38

present moment 3, 6, 8-9, 20, 24, 28, 35, 40-41, 47
psychological flexibility 3, 6, 18, 25, 50

self-as-context 6, 20, 23, 47

values 3, 6, 8-9, 18, 20-21, 25-27, 29, 31, 38, 39, 41-42, 44, 48

young adults 1-3, 11

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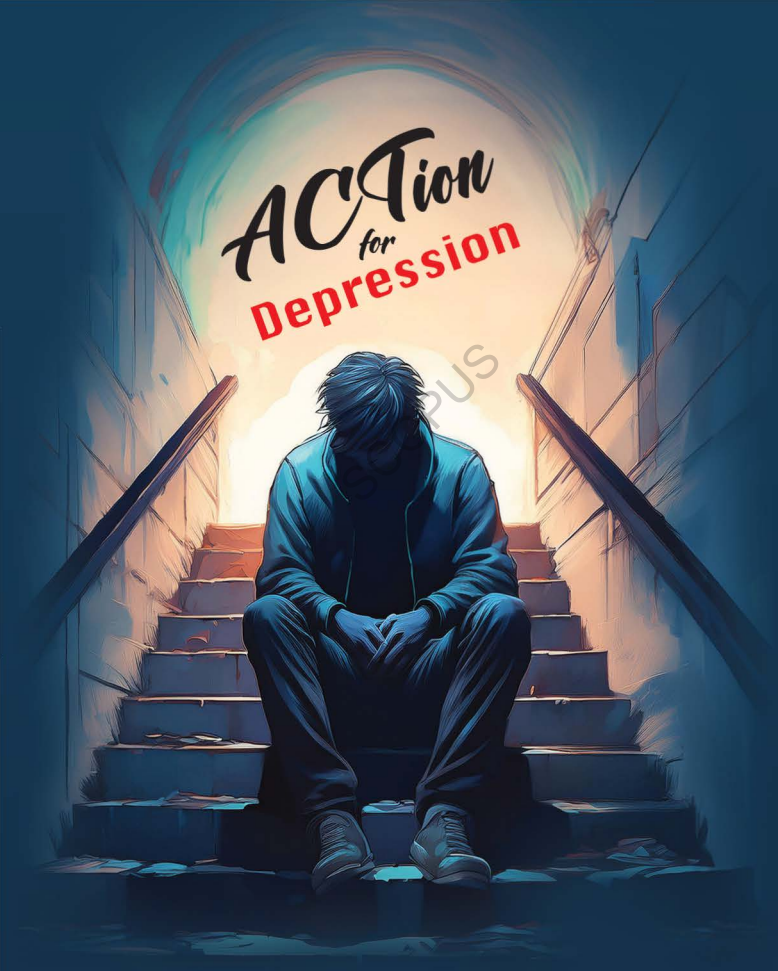

Struggling with depression can feel like an unending battle. "ACTion for Depression" offers a new way forward, combining the science of Acceptance and Commitment Therapy (ACT) with compassionate, real-world guidance. This book provides a roadmap for those seeking to understand and manage depression, focusing on acceptance, mindfulness, and values-based action.

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43400 UPM Serdang
Selangor Darul Ehsan

Tel: 03-9769 8851 / 8429
Fax: 03-9769 3623
Website: https://penerbit.upm.edu.my
E-mail: penerbit@upm.edu.my

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